

بِسْمِ اللَّهِ الرَّحْمَنِ الرَّحِيمِ



MMPI Test

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MINNESOTA MULTIPHASIC PERSONALITY INVENTORY- MMPI

- ▶ The Minnesota Multiphasic Personality Inventory (MMPI; Hathaway & McKinley, 1943) has been the most widely used self-report measure of adult personality and psychopathology (Lubin, Larsen, & Matarazzo, 1984).
- ▶ It is a written form of standardized clinical assessment tool, an objective test, used to diagnose psychological problems.
- ▶ The MMPI is copyrighted by the [University of Minnesota](#).
- ▶ is a standardized questionnaire that elicits a wide range of self-descriptions
- ▶ is a true /false self-report questionnaire. Statements are typically of the self-reference type such as “I like good food” and “I never have trouble falling asleep.”
- ▶ Subjects mark “True” or “False” for each statement as it applies to themselves

Purpose of DEVELOPMENT

- ❑ The original development of the MMPI was begun in 1939 at the University of Minnesota by Starke R. Hathaway and J. Charnley McKinley.
- ❑ They wanted an instrument that could serve as an aid in assessing adult patients during routine psychiatric case workups and that could accurately determine the severity of their disturbances.
- ❑ Hathaway and McKinley were interested in developing an objective estimate of the change produced by psychotherapy or other variables in a patient's life.

Advantages: Professionals use various versions of the MMPI to

- ❑ Identify and quantify any existing psychopathology
- ❑ help develop treatment plans,
- ❑ gives a quantitative measurement of an individual's level of Emotional adjustment and Attitude towards test taking
- ❑ assist with differential diagnosis,
- ❑ help answer legal questions (forensic psychology),
- ❑ screen job candidates during the personnel selection process,
- ❑ as part of a therapeutic assessment procedure

- ❑ useful in providing information in several areas:
- ❑ attitudes toward assessment,
- ❑ cooperation,
- ❑ cognitive/ideation,
- ❑ mood,
- ❑ conflict areas,
- ❑ coping styles,

The original 10 clinical scales designed were:

No.	Abbreviation	Description	What is measured	No. of items
1	Hs	<u>Hypochondriasis</u>	Concern with bodily symptoms	32
2	D	<u>Depression</u>	Depressive Symptoms	57
3	Hy	<u>Hysteria</u>	Awareness of problems and vulnerabilities	60
4	Pd	<u>Psychopathic Deviate</u>	Conflict, struggle, anger, respect for society's rules	50
5	MF	<u>Masculinity/Femininity</u>	Stereotypical masculine or feminine interests/behaviors	56
6	Pa	<u>Paranoia</u>	Level of trust, suspiciousness, sensitivity	40
7	Pt	<u>Psychasthenia</u>	Worry, Anxiety, tension, doubts, obsessiveness	48
8	Sc	<u>Schizophrenia</u>	Odd thinking and social alienation	78
9	Ma	<u>Hypomania</u>	Level of excitability	46
10	Si	<u>Social Introversion</u>	People orientation	69

Range of problems that the MMPI and MMPI-2 can help with are:

- ❑ eating disorders,
- ❑ soldiers' reaction in battle,
- ❑ posttraumatic stress syndrome,
- ❑ the detection of sexual abuse in children,
- ❑ risk factors of female criminals,
- ❑ the effects of acculturation,
- ❑ differentiating criminal types,
- ❑ prediction of delinquent behavior,
- ❑ prediction of neurological disorders,
- ❑ psychosis-prone college students

HISTORY OF THE MMPI

- ❑ 1)Originally developed by Starke R. Hathaway and J. C. McKinley,
- ❑ at University of Minnesota,
- ❑ first published by the University of Minnesota Press in 1943
- ❑ Beginning with a pool of 1000 items selected from a wide variety of sources, including case histories, psychological reports, textbooks, and existing tests, Initial Item pool covered contents and themes from a wide range of psychiatric diagnoses
- ❑ empirical criterion keying: scales were determined empirically by presenting the items to criterion and control groups. This refers to the development, selection, and scoring of items within the scales based on some external criterion of reference
- ❑ To use an empirical [criterion] keying, the criterion groups consisted of psychiatric inpatients at the University of Minnesota Hospital
- ❑ then compared with control group consisting primarily of relatives and visitors of the patients, excluding mental patients, in the University of Minnesota Hospital, and a group of recent high-school graduates

- ❑ developed item clusters with maximum discriminant validity by comparing psychiatric patients with "normal" persons (control group).
- ❑ Despite its weakness, the original control group did provide a reference sample.
- ❑ The original MMPI contained 566 items
- ❑ After an item analysis was conducted, items that separated the criterion from the control group were included in diagnostic scales.
- ❑ To cross-validate the scales, independent samples of the criterion and control groups were administered the items. To qualify as cross-validated, a scale had to distinguish the criterion group from the control group at the .05 level of significance (i.e., the probability of obtaining differences by chance is less than 5 out of 100).

Initial Interpretations

- ❑ To cross-validate the scales, independent samples of the criterion and control groups were administered the items.
- ❑ To qualify as cross-validated, a scale had to distinguish the criterion group from the control group at the .05 level of significance (i.e., the probability of obtaining differences by chance is less than 5 out of 100).
- ❑ For all of the scales, the control group provided the reference for which standard scores were determined.
- ❑ McCall's T, with a *mean of 50 and a standard deviation of 10*, was used to compute standard scores.

The Re-standardization: MMPI-2

- ❑ With the new norms for the MMPI-2, T scores of 65 are now considered significant.
- ❑ To update and restandardize the MMPI, the MMPI-2 (Butcher, Graham, Dahlstrom, Tellegen, & Kaernmer, 1989) was worked out.
- ❑ The MMPI-2 has 567 items.
- ❑ The purpose of the revision was to update and expand the norms; revise items that were out of date, awkward, outdated, or problematic; simplified these items and broaden the item pool to extend the range of constructs that one could evaluate.

MMPI II Scales

- ❑ In the United States the Minnesota Multiphasic Personality Inventory (MMPI; Hathaway & McKinley, 1943) has been the most widely used self-report measure of adult personality and psychopathology (Lubin, Larsen, & Matarazzo, 1984). The MMPI has been translated into more than 115 languages for use in 46 countries (Ben-Porath, 1990; Butcher, 1985) and has demonstrated external validity in a wide variety of national and international settings
- ❑ The MMPI-2 consists of 9 validity and 10 clinical scales in the basic profile, along with 15 content scales, 9 restructured clinical scales, and 20 supplementary scales

6 Goals of Re-standardization

- ❑ 1. The deletion of obsolete or objectionable items.
- ❑ 2. Continuation of the original validity and clinical scales.
- ❑ 3. The development of a wide, representative normative sample.
- ❑ 4. Norms that would most accurately reflect clinical problems and would result in a uniform percentile classification.
- ❑ 5. The collection of new clinical data that could be used in evaluating the items and scales.
- ❑ 6. The development of new scales.

Revision and Re-standardization of the MMPI-1 leading to MMPI-2

- ❑ 2)replaced by an updated version, the MMPI-2, in 1989 (Butcher, Dahlstrom, Graham, Tellegen, and Kraemmer).^[4]
- ❑ 3)A version for adolescents, the MMPI-A, was published in 1992.
- ❑ An alternative version of the test, the MMPI-2 Restructured Form (MMPI-2-RF), published in 2008,
- ❑ retains some aspects of the traditional MMPI assessment strategy, but adopts a different theoretical approach to personality test development
- ❑ Interpretation of the clinical scales remained the same.
- ❑ the scales were re-normed and scores transformed to uniform T scores.

MMPI-2 Content Scales

- ❑ The MMPI-2 contains 15 content scales, including HEA (health concerns) and TPA, which evaluates for the hard driving, irritable, impatient Type A personality.
- ❑ Other MMPI-2 content scales include FAM (family problems), which evaluates family disorders and possible child abuse, and WRK (work interference), which examines behaviors or attitudes likely to interfere with work performance.
- ❑ original MMPI had 13 standard scales, of which 3 related to validity and 10 related to clinical or personality indices. The more recent MMPI-2 and MMPI-A have maintained the original 10 clinical/personality scales as well as the original 3 validity scales, but the total number of validity scales has been increased

- ❑ The clinical and personality scales are known both by their scale numbers and by scale abbreviations.
- ❑ These include scales based on item content (content scales),
- ❑ subscales for the clinical and personality scales based on clusters of content-related items (Harris-Lingoes subscales),
- ❑ assessment of items and item clusters that relate to relevant dimensions (critical items)
- ❑ empirically derived new scales (supplementary scales).
- ❑ **MMPI-Adolescent (MMPI-A)**, first made available in 1992 (Butcher et al., 1992) used for adolescents age 14 to 18, contains 478 items, a four factor model (similar to all of the MMPI instruments) was chosen for the MMPI-A included
 - ❑ General Maladjustment,
 - ❑ Over-control (repression) (L, K, Ma),
 - ❑ Si (Social Introversion),
 - ❑ MF (Masculine/Feminine)

❑ 4)The MMPI-2-RF

- ❑ The MMPI-2-RF builds on the Restructured Clinical (RC) scales developed in 2003,^[7] and subsequently subjected to extensive research,^[28] with an overriding goal of improved discriminant validity, or the ability of the test to reliably differentiate between clinical syndromes or diagnoses.

❑ 5)MMPI-A-RF

- ❑ The Minnesota Multiphasic Personality Inventory - Adolescent - Restructured Form (MMPI-A-RF) is a broad-band instrument used to psychologically evaluate adolescents.^[45] It was published in 2016

The RC scales measure the core constructs of the original clinical scales.



Scale	Abbreviation	Description	What is measured
RCd	dem	Demoralization	A general measure of distress that is linked with anxiety, depression, helplessness, hopelessness, low self-esteem, and a sense of inefficacy ^[34]
RC1	som	Somatic Complaints	Measures an individual's tendency to medically unexplainable physical symptoms ^[34]
RC2	lpe	Low Positive Emotions	Measures features of anhedonia - a common feature of depression ^[34]
RC3	cyn	Cynicism	Measures a negative or overly-critical worldview that is associated with an increased likelihood of impaired interpersonal relationships, hostility, anger, low trust, and workplace misconduct ^[34]
RC4	asb	Antisocial Behavior	Measures the acting out and social deviance features of antisocial personality such as rule breaking, irresponsibility, failure to conform to social norms, deceit, and impulsivity that often manifests in aggression and substance abuse ^[24]

The RC scales measure the core constructs of the original clinical scales.

Scale	Abbreviation	Description	What is measured
RC6	per	Ideas of Persecution	Measures a tendency to develop paranoid delusions, persecutory beliefs, interpersonal suspiciousness and alienation, and mistrust ^[34]
RC7	dne	Dysfunctional Negative Emotions	Measures a tendency to worry/be fearful, be anxious, feel victimized and resentful, and appraise situations generally in ways that foster negative emotions ^[34]
RC8	abx	Aberrant Experiences	Measures risk for psychosis, unusual thinking and perception, and risk for non-persecutory symptoms of thought disorders ^[34]
RC9	hpm	Hypomanic Activation	Measures features of mania such as aggression and excitability ^[34]

Content Scales

Abbr.	Description	What is measured?
ANX	Anxiety	General symptoms of anxiety, somatic problems, nervousness or worry
FRS	Fears	Specific fears and general fearfulness
OBS	Obsessiveness	Difficulty making decisions, excessive rumination and dislike change
DEP	Depression	Feelings of low mood, lack of energy, suicidal ideation and other depressive features
HEA	Health Concerns	Concerns about illness and physical symptoms
BIZ	Bizarre Mentation	The presence of psychotic thought processes
ANG	Anger	Feelings and expression of anger
CYN	Cynicism	Distrust and suspiciousness of other people and their motives

Content scales

Abbr.	Description	What is measured?
ASP	Antisocial Practices	Expression of nonconforming attitudes and possible issues with authority
TPA	Type A Behavior	Irritability, impatience and competitiveness
LSE	Low Self Esteem	Negative attitudes about self, own ability and submissiveness
SOD	Social Discomfort	Preferring to be alone and discomfort when meeting new people
FAM	Family Problems	Resentment, anger and perceived lack of support from family members
WRK	Work Interference	Attitudes that contribute to poor work performance
TRT	Negative Treatment Indicators	Feelings of pessimism and unwillingness to reveal personal information to others
ASP	Antisocial Practices	Expression of nonconforming attitudes and possible issues with authority
TPA	Type A Behavior	Irritability, impatience and competitiveness

Supplemental Scales: Some Samples:

Abbreviation	Description
Es	Ego Strength Scale
OH	Over-Controlled Hostility Scale
MAC	MacAndrews Alcoholism Scale
MAC-R	MacAndrews Alcoholism Scale Revised
Do	Dominance Scale
APS	Addictions Potential Scale
AAS	Addictions Acknowledgement Scale
SOD	Social Discomfort Scale
A	Anxiety Scale
R	Repression Scale
TPA	<u>Type A</u> Scale
MDS	Marital Distress Scale

The Content Scales

- ❑ contain items intended to provide insight into specific types of symptoms and areas of functioning that the clinical scales do not measure; and are supposed to be used in addition to the clinical scales to interpret profiles
- ❑ susceptible to response bias - exaggeration or denial of symptoms, and should be interpreted with caution.
- ❑ T scores greater than 65 on any content scale are considered high scores.

RELIABILITY AND VALIDITY

- ▶ Reliability studies on the original MMPI indicate that it had moderate levels of temporal stability and internal consistency.
- ▶ Hunsley, Hanson, and Parker (1988) completed a meta-analysis of studies performed on the MMPI between 1970 and 1981, and concluded, “all MMPI scales are quite reliable, with values that range from a low of .71 (Scale Ma) to a high of .84 (Scale Pt)” (p. 45).
- ▶ Reliability reported in the MMPI-2 manual indicates moderate test-retest reliabilities. Where as test retest reliability coefficient range from the .50 to .90
- ▶ The different scales correlate highly,
- ▶ studies provide extensive evidence of the MMPI’s construct validity.
- ▶ During interpretation it is important to consider the subject’s demographic characteristics when interpreting profiles .i.e, age, gender, race, place of

residence, and other demographic characteristics, education, income, and socioeconomic status



VALIDITY SCALES

Abbreviation	New in version	Description	Assesses
CNS	1	"Cannot Say"	Questions not answered
L	1	Lie	Client "faking good"
F	1	Infrequency	Client "faking bad" (in first half of test)
K	1	Defensiveness	Denial/Evasiveness
Fb	2	Back F	Client "faking bad" (in last half of test)
VRIN	2	Variable Response Inconsistency	answering similar/opposite question pairs inconsistently
TRIN	2	True Response Inconsistency	answering questions all true/all false
F-K	2	F minus K	honesty of test responses/not faking good or bad
S	2	Superlative Self-Presentation	improving upon K scale, "appearing excessively good"
Fp	2	F-Psychopathology	Frequency of presentation in clinical setting
Fs	2 RF	Infrequent Somatic Response	Over reporting of somatic symptoms

? = Cannot Say Scale

- ❑ This scale adds how many questions were left unanswered.
- ❑ A high amount of blank responses may signal confusion, resistance to taking the test, or simply that they did not finish.
- ❑ More than 10 omitted answers risks rendering invalid the totality of the results
- ❑ Measures the tendency to respond inconsistently.
- ❑ There are questions in the MMPI-2 that repeat using different wording.
- ❑ This scale scores the consistency of the answers.
- ❑ On the one hand, an elevated **VRIN** and **F** indicate that the person answered questions at random; thus, *the profile is invalid*

F = Infrequency Scale

- ❓ This quantifies how much a person's responses deviate from the general population; hence, how infrequent answers are when compared to everyone else.
- ❓ In a non-clinical setting, a **T-Score above 80** on this scale probably **evidences the existence of a severe psychopathology**.
- ❓ If **F** scores are low, they indicate "**Fake-Bad**" behavior (trying to appear worse than one is).
- ❓ If **F** scores are high, they indicate "**Fake Good**" behavior (trying to appear better than one is).

K = Correction Scale

- ❑ This scale measures defensiveness in a much more subtle way than Lie.
- ❑ T-Scores between a 55 and 70, signifies that individual is competent in managing his or her life
- ❑ A low T-score of 45 or less hints that a psychopathology is probably present
- ❑ Interestingly, a really low **K** of 35 or less correlates with a poor prognosis because it signals that the test taker does not have the tools or the psychological strength to respond well to traditional (no-drugs) therapies, most likely lacking sufficient Ego-Strength (**Es**).

ADMINISTRATION

- ❑ The MMPI-2 can be administered to persons who are 16 years of age or older with an eighth-grade reading level.
- ❑ have only one booklet form, although these are available in either softcover or hardcover.
- ❑ A separate answer sheet suitable for both individual examinations and group testing programs.
- ❑ statements covering a wide range of subject matter from the physical condition to the morale and the social attitudes of the individual being tested.
- ❑ The subject is asked to sort all the statements into three categories, True, False, and Cannot Say.
- ❑ is completed by most persons in 1 to 1½ hours: 90 minutes

THE BOOKLET FORMS

- The booklets must be clean and complete.
- The marks should be erased before the booklets are reused.
- If the answer sheet are to be scored by machine, pencils appropriate for the machine should be provided. Even if the responses are to be scored by hand, pencils should be supplied; marking with a pen does not permit.
- The session should be started with a brief statement of what will be done and the purpose of the testing.
- The information requested includes the subject name, sex, and other data such as age, schooling, and home address that are needed to identify the subject or group.

- ❓ examiners should explain to clients the reason for testing and how the results will be used. It might also be pointed out that the test was designed to determine whether someone has presented himself or herself in an either unrealistically positive or exaggeratedly disturbed manner. Thus, the best strategy is to be as honest and as clear as possible. Finally, it might be clarified that some, or even many, of the questions might seem a bit unusual. They have been developed to assess individuals with a wide range of personality styles and problem presentations. If they don't apply to the person taking the test, this should be indicated with either a true or false response. I

Test Instructions

- When form R is being administered, tell the subject to fold back the cover and turn to the answer sheet.
- Tell him to enter his name in the spaces at the top on the answering sheet before beginning the inventory.
- Then tell the subject to read the instructions on the first page of the test booklet, or read them aloud to him.
- The subjects should be encouraged to answer every item.
- The instructions on the booklet forms include the statements, Do not leave any blank spaces.
- Try to make some answer to every statement.
- Do not make many (Cannot Say) responses.
- Do not leave any blank spaces if you can make any judgment regarding your answers.

Types of Administration

- ❑ An online computer administration is available through National Computer Systems.
- ❑ For persons who have special difficulties, an individual (Box) form and a tape-recorded form have been developed.
- ❑ The Box form is most appropriate for persons who have difficulties concentrating and/or reading. Each item is presented on a card, which the person is requested to place into one of three different sections to indicate a “true,” “false,” or “cannot say” response.
- ❑ The tape recorded form is used for persons who have reading difficulties because of factors such as illiteracy, blindness, or aphasia

Bubble Sheet For Computerized Scoring

MINNESOTA MULTIPHASE PERSONALITY INVENTORY

Dept. of Psychology
FC -R

DIRECTIONS FOR MARKING NAME SLIP
Print your name in the boxes above the columns of letters.
Print your last name first. Skip a box then print as much of
your first name as possible. Below each box blacken the
circle that is lettered the same as the letter in the box.
Blacken the blank circle for spaces.

Age: Do not write below unless told to.

MALE
FEMALE

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70-220AS

INTERPRETATION PROCEDURE

- Very little instruction and supervision are required.
- Follow the key from the manual to discover correct responses on each subscales., in manual scoring.

Mention raw scores on the scoring sheet

- The administrators materials include Hand-scoring or Machine-scoring keys and profile forms.
- Several types of answer sheets have been prepared for use with the group form booklet.
- The answer sheet for the IBM 805 test scoring machine may also be scored by hand.
- Another answer sheet has been designed for use with either the IBM 1230 or 1232 machine.

INTERPRETATION PROCEDURE

- ❑ Step 1. Completion Time
- ❑ Step 2. Score and Plot the Profile Complete the scoring and plot the profile. Specific directions for tabulating the MMPI2 raw scores and converting them into profiles are provided in Appendix G on page 690.

Step 3. Organize the Scales and Identify the Code Type

- The scores can be summarized by simply listing the scores according to the order in which they appear on the profile sheet (L, F, K, 1, 2, 3, etc.) with their T scores to the right of these scales. For the purposes of communicating scale scores, T scores rather than raw scores should be used. Developing summary codes (“code types”) provides a shorthand method of recording MMPI-2/MMPI-A results. Code types can be determined by simply looking at the two highest scale elevations. For example, the two highest scores in a profile might be 8 and 7 resulting in an 87/78 code type. The 87/78 code type can then be looked up in the TwoPoint Codes section to obtain various descriptions relating to that code type

Step 4. Determine Profile Validity

- ▶ Assess the validity of the profile by noting the pattern of the validity scales. There are a number of indicators suggesting invalid profiles, which are described in the next section. However, the basic patterns include a defensive style in which pathology is minimized (elevated L and/or K on the MMPI-2 and MMPI-A), an exaggeration of pathology (elevated F and/or Fb on the MMPI-2 or F, F1, or F2 on the MMPI-A), or an inconsistent response pattern (elevated VRIN or TRIN). In addition, clinicians should consider the context of the assessment to determine whether a defensive, fake bad, or inconsistent response style supports what is known about the client. In particular, the examiner should determine the likelihood that the examinee would potentially gain by over- or underreporting psychopathology

- ❓ Step 5. Determine Overall Level of Adjustment Note the number of scales over 65 and the relative elevation of these scales. The degree to which F is elevated can also be an excellent indicator of the extent of pathology (assuming that it is not so high as to indicate an invalid profile). The greater the number and relative elevation of these scales, the more the individual is likely to have difficulties carrying out basic responsibilities and experience social and personal discomfort.

- ❓ Step 6. Describe Symptoms, Behaviors, and Personality Characteristics
- ❓ Mild elevations on individual scales (T = 60-65) represent tendencies or trends in the individual's personality.
- ❓ Elevations above 65 on the MMPI-2 and MMPI-A are more strongly characteristic of the individual and, with progressively greater increases, are more likely to represent core features of personality functioning. However, basing interpretations solely on specific T score elevations may be misleading because a client's demographic characteristics often exert a strong influence.

Some examples:

- ❑ Anxiety. Elevations on Scale 7 (Pt), especially if 7 (Pt) is greater than 8 (Sc), suggest anxiety.
- ❑ Depression. A high score on 2 (D) combined with a low score on 9 (Ma) is particularly indicative of depression.
- ❑ Mania. A high score on 9 (Ma) combined with a low score on 2 (D) suggests mania.
- ❑ Psychosis. A high score on 8 (Sc), especially if 8 (Sc) is 10 points or more higher than 7 (Pt), suggests psychosis.

NAME

LAST

FIRST

MIDDLE

AGE

SEX

B. 057.

K = + = -
F = H = P =
H = H = P =
D = M =

P = + = -
P = + = -
S = + = -
M = + = -

DATE

THE MINNESOTA MULTIPHASE PERSONALITY INVENTORY, by Hathaway and McKinley

SCORES

1	T	F	31	T	F	61	T	F	91	T	F	121	T	F	151	T	F	181	T	F	211	T	F	241	T	F	271	T	F
2			32			62			92			122			152			182			212			242			272		
3			33			63			93			123			153			183			213			243			273		
4			34			64			94			124			154			184			214			244			274		
5			35			65			95			125			155			185			215			245			275		
6	T	F	36	T	F	66	T	F	96	T	F	126	T	F	156	T	F	186	T	F	216	T	F	246	T	F	276	T	F
7			37			67			97			127			157			187			217			247			277		
8			38			68			98			128			158			188			218			248			278		
9			39			69			99			129			159			189			219			249			279		
10	T	F	40	T	F	70	T	F	100	T	F	130	T	F	160	T	F	190	T	F	220	T	F	250	T	F	280	T	F
11			41			71			101			131			161			191			221			251			281		
12			42			72			102			132			162			192			222			252			282		
13			43			73			103			133			163			193			223			253			283		
14			44			74			104			134			164			194			224			254			284		
15			45			75			105			135			165			195			225			255			285		
BE SURE YOUR MARKS ARE HEAVY AND BLACK. ERASE COMPLETELY ANY ANSWER YOU WISH TO CHANGE.																													
16	T	F	46	T	F	76	T	F	106	T	F	136	T	F	166	T	F	196	T	F	226	T	F	256	T	F	286	T	F
17			47			77			107			137			167			197			227			257			287		
18			48			78			108			138			168			198			228			258			288		
19			49			79			109			139			169			199			229			259			289		
20	T	F	50	T	F	80	T	F	110	T	F	140	T	F	170	T	F	200	T	F	230	T	F	260	T	F	290	T	F
21			51			81			111			141			171			201			231			261			291		
22			52			82			112			142			172			202			232			262			292		
23			53			83			113			143			173			203			233			263			293		
24			54			84			114			144			174			204			234			264			294		
25	T	F	55	T	F	85	T	F	115	T	F	145	T	F	175	T	F	205	T	F	235	T	F	265	T	F	295	T	F
26			56			86			116			146			176			206			236			266			296		
27			57			87			117			147			177			207			237			267			297		
28			58			88			118			148			178			208			238			268			298		
29			59			89			119			149			179			209			239			269			299		
30			60			90			120			150			180			210			240			270			300		

scoring sheet for Urdu version of MMPI

																		?
1	26	51	76	101	126	151	176	201	226	251	276	301	326	351	376			L
2	27	52	77	102	127	152	177	202	227	252	277	302	327	352	377			F
3	28	53	78	103	128	153	178	203	228	253	278	303	328	353	378			K
4	29	54	79	104	129	154	179	204	229	254	279	304	329	354	379			1
5	30	55	80	105	130	155	180	205	230	255	280	305	330	355	380			Hs
6	31	56	81	106	131	156	181	206	231	256	281	306	331	356	381			2
7	32	57	82	107	132	157	182	207	232	257	282	307	332	357	382			D
8	33	58	83	108	133	158	183	208	233	258	283	308	333	358	383			3
9	34	59	84	109	134	159	184	209	234	259	284	309	334	359	384			Hy
10	35	60	85	110	135	160	185	210	235	260	285	310	335	360	385			4
11	36	61	86	111	136	161	186	211	236	261	286	311	336	361	386			Pd
12	37	62	87	112	137	162	187	212	237	262	287	312	337	362	387			5
																		Mf
13	38	63	88	113	138	163	188	213	238	263	288	313	338	363	388			6
14	39	64	89	114	139	164	189	214	239	264	289	314	339	364	389			Pa
15	40	65	90	115	140	165	190	215	240	265	290	315	340	365	390			7
16	41	66	91	116	141	166	191	216	241	266	291	316	341	366	391			Pt
17	42	67	92	117	142	167	192	217	242	267	292	317	342	367	392			8
18	43	68	93	118	143	168	193	218	243	268	293	318	343	368	393			Sc
19	44	69	94	119	144	169	194	219	244	269	294	319	344	369	394			9
20	45	70	95	120	145	170	195	220	245	270	295	320	345	370	395			Ma
21	46	71	96	121	146	171	196	221	246	271	296	321	346	371	396			0
22	47	72	97	122	147	172	197	222	247	272	297	322	347	372	397			Si
23	48	73	98	123	148	173	198	223	248	273	298	323	348	373	398			
24	49	74	99	124	149	174	199	224	249	274	299	324	349	374	399			
25	50	75	100	125	150	175	200	225	250	275	300	325	350	375				

INTERPRETATION: Psychologist writes up a report while interpreting the test results.

- ▶ computerized interpretation systems available is “The Minnesota Report™ (Butcher, 1993) is the best. This system generates a very cautious and methodical 16-page report that includes discussion of profile validity, symptomatic patterns, interpersonal relations, diagnostic considerations, and treatment considerations.
- ▶ The first step in interpreting any personality test profile is to establish its validity to assure the subject's cooperativeness in taking the test.
- ▶ The obtained raw scores are used then for plotting individual profiles.
- ▶ The following eight steps are recommended for interpreting MMPI-2/MMPI-A profiles. These steps should be followed with a knowledge and awareness of

The implications of demographic variables on the cultural, intellectual
level, education, social class and occupation of the population



L		F		K		1 - Hs		2 - D		3 - Hy		4 - Pd	
T	F	T	F	T	F	T	F	T	F	T	F	T	F
	15	14	17	96	30	23	3	5	2	10	2	16	8
	30	27	20		39	29	3	32	8	23	3	21	20
	45	31	54		71	43	7	41	9	32	6	24	37
	60	34	65		89	62	9	43	18	43	7	32	82
	75	35	75		124	72	18	52	30	44	8	33	91
	90	40	83		129	108	51	67	36	47	9	35	96
	105	42	112		134	114	55	86	39	76	12	38	107
	120	48	113		138	125	63	104	46	114	26	42	134
	135	49	115		142	161	68	130	51	179	30	61	137
	150	50	164		148	189	130	138	57	186	51	67	141
	165	53	169		160	273	153	142	58	189	55	84	155
	195	56	177		170		155	158	64	238	71	94	170
	225	66	185		171		163	159	80	253	89	102	171
	253	85	196		180		175	182	88		93	106	173
	283	121	199		183		188	189	89		103	110	180
		123	220		217		190	192	95		107	118	183
		139	257		234		192	236	98		109	127	201
		146	258		267		230	259	107		124	215	231
		151	272		272		243	288	122		128	216	235
		156	276		296		274	290	131		129	224	237
		168			316		231		145		136	239	248
		184			322				152		137	244	267
		197			368				153		141	245	287
		200			370				154		147	284	289
		202			372				155		153		294
		205			373				160		160		296
		206			375				178		162		
		209			386				191		163		
		210			394				207		170		
		211							208		172		
		215							233		174		
		218							241		175		
		227							242		180		
		245							248		188		
		246							263		190		
		247							270		192		
		252							271		201		
		256							272		213		
		269							285		230		
		275							296		234		
		286									243		
		288									265		
		291									267		
		293									274		
											279		
											289		
											292		
0	15	44	20	1	29	11	21	20	40	13	47	24	26

Step 1. Completion Time

- ▶ The examiner should note the length of time required to complete the test. For a mildly disturbed person who is 16 years or older with an average IQ and eighth-grade education, the total completion time for the MMPI-2 should be approximately 90 minutes.
- ▶ Computer administrations are usually 15 to 30 minutes shorter (60 to 75 minutes in total). The MMPI-A usually takes 60 minutes to complete with computer administrations taking 15 minutes less time (45 minutes in total).
- ▶ If two or more hours are required for the MMPI-2 or 1.5 or more for the MMPI-A, the following interpretive possibilities must be considered: • Major psychological disturbance, particularly a severe depression or functional psychosis. • Obsessive indecision. • Below-average IQ or poor reading ability

resulting from an inadequate education system. • General Learning Improvement Program



Completion Time

- ❑ If, on the other hand, an examinee finishes in less than an hour, the examiner should suspect an invalid profile, an impulsive personality, or both.
- ❑ Note any erasures or pencil points on the answer sheet.
- ❑ The presence of a few of these signs may indicate that the person took the test seriously and reduces the likelihood of random marking;
- ❑ a great number of erasures may reflect obsessive-compulsive tendencies.
- ❑ Step 2. Score and Plot the Profile: Complete the scoring and plot the profile. Specific directions for tabulating the MMPI2 raw scores and converting them into profiles.

Table No. 1

Fractional Raw Scores For Any Given Raw Score of K
Fractions of Raw K

Raw K	$\frac{K}{5}$	$\frac{4K}{5}$	$\frac{2K}{5}$
30	15	12	6
29	15	12	6
28	14	11	6
27	14	11	5
26	13	10	5
25	13	10	5
24	12	10	5
23	12	9	5
22	11	9	4
21	11	8	4
20	10	8	4
19	10	8	4
18	9	7	4
17	9	7	3
16	8	6	3
15	8	6	3
14	7	6	3
13	7	5	3
12	6	5	2
11	6	4	2
10	5	4	2
9	5	4	2
8	4	3	2
7	4	3	1
6	3	3	1
5	3	2	1
4	2	2	1
3	2	1	1
2	1	1	0
1	1	1	0
0	0	0	0

MMPI-2™ Minnesota Multiphasic Personality Inventory-2™

Profile for Validity and Clinical Scales

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Name _____

Address _____

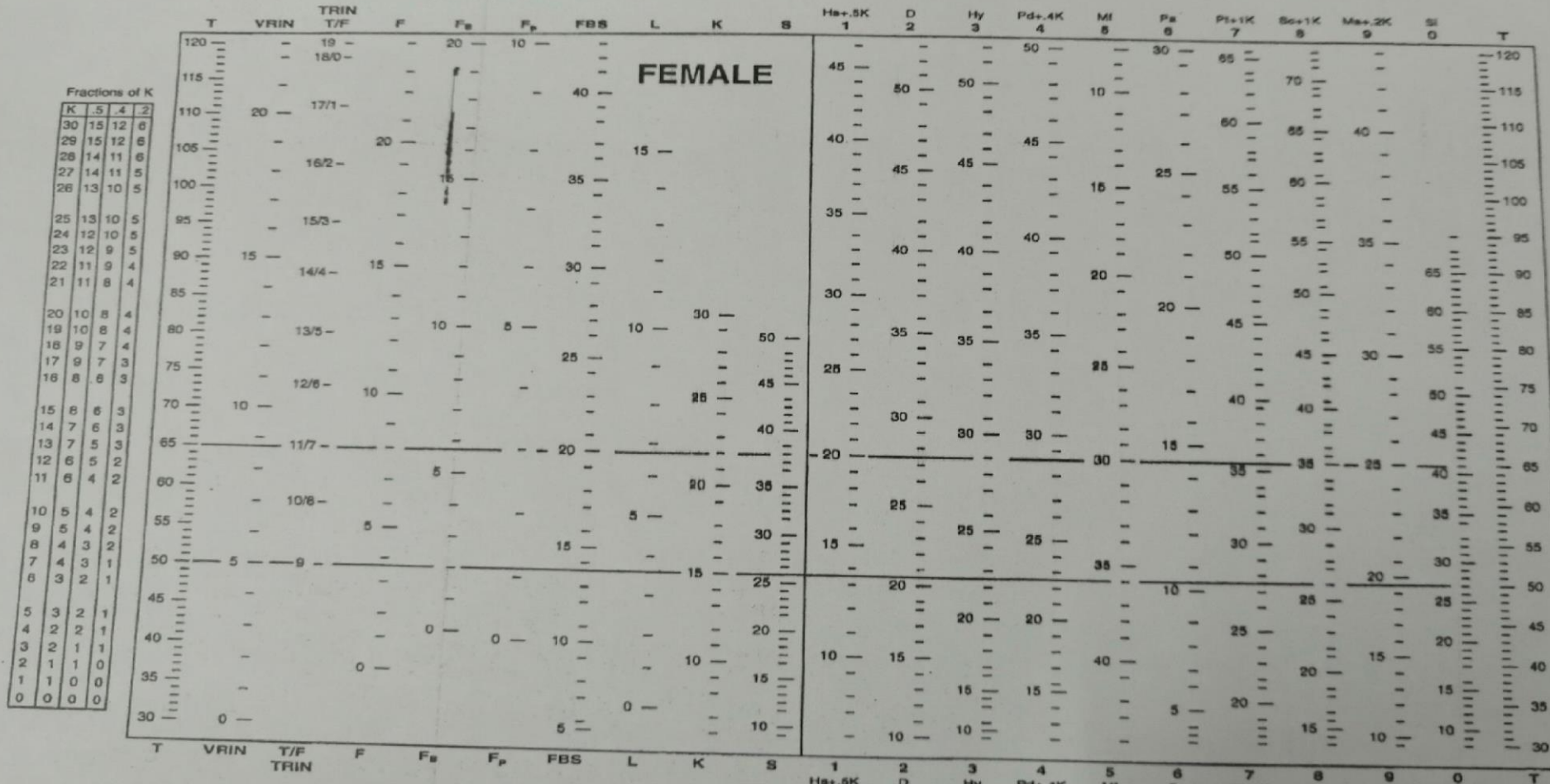
Occupation _____ Date Tested _____

Education _____ Age _____ Marital Status _____

Referred by _____

MMPI-2 Code _____

Scorer's Initials _____



MMPI-2

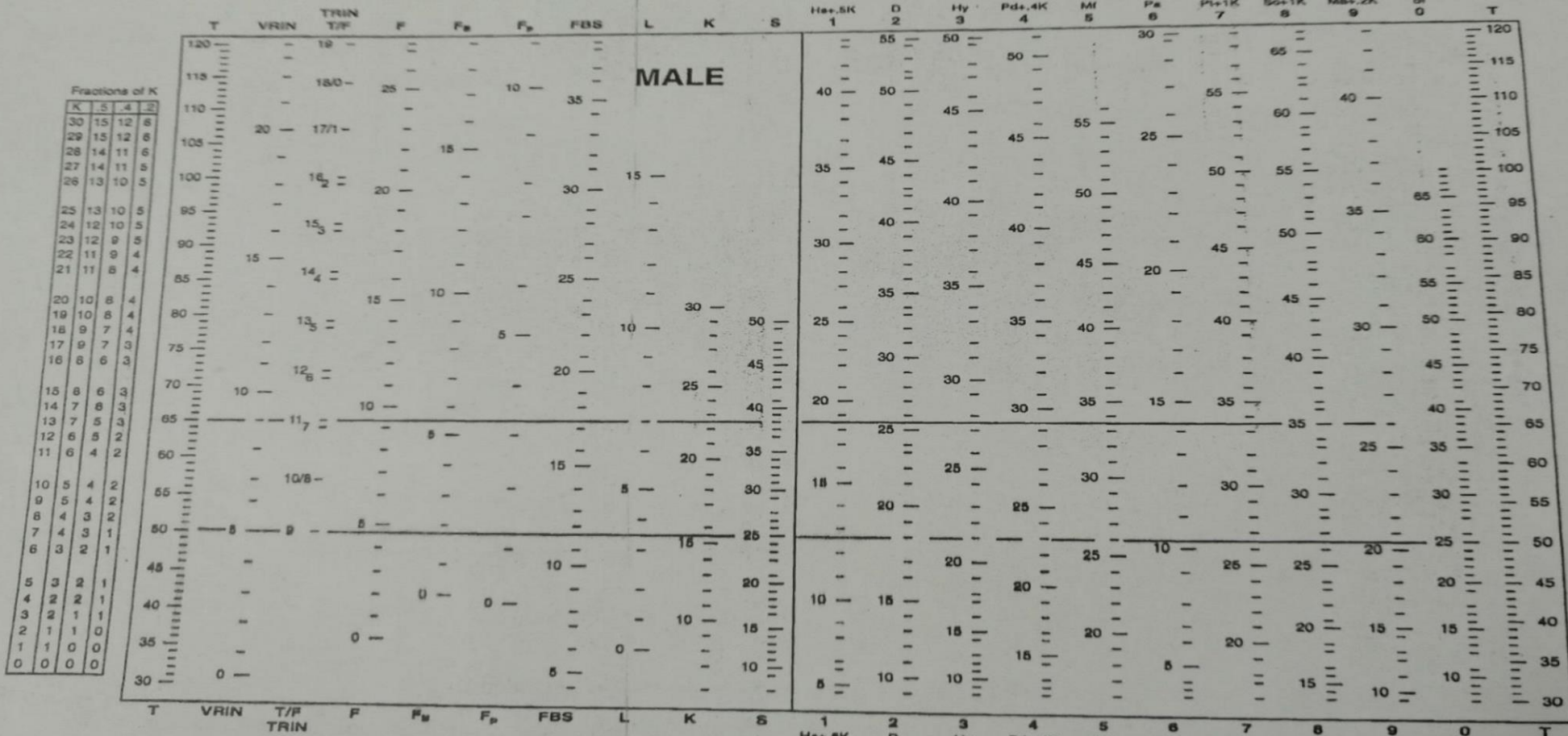
Minnesota Multiphasic
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Profile for Validity and Clinical Scales

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Name _____
Address _____
Occupation _____ Date Tested _____
Education _____ Age _____ Marital Status _____
Referred by _____
MMPI-2 Code _____

Scorer's Initials _____



Raw Score _____

? Raw Score _____

K to be Added _____

Raw Score with K _____

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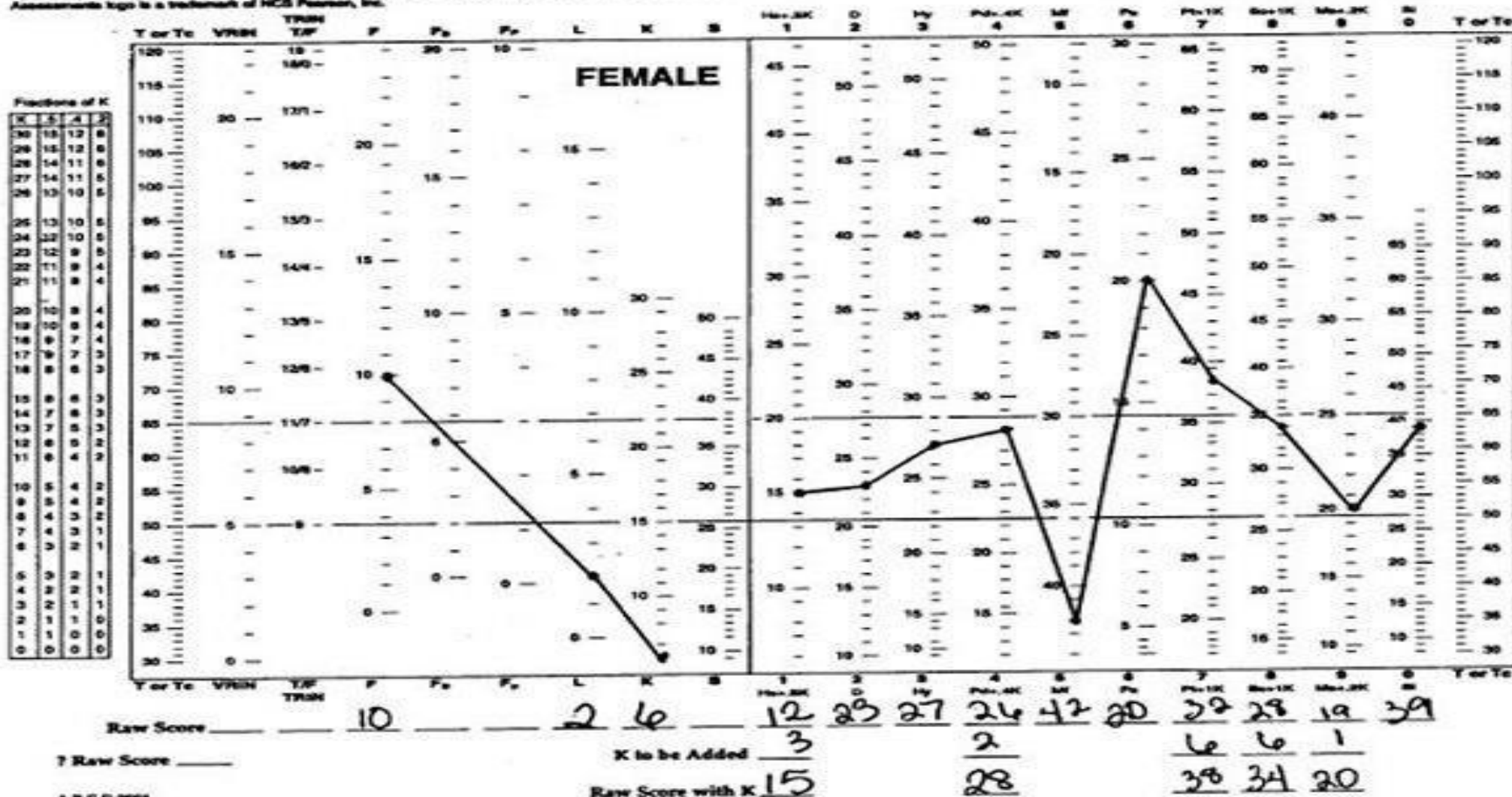
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Profile for Validity and Clinical Scales

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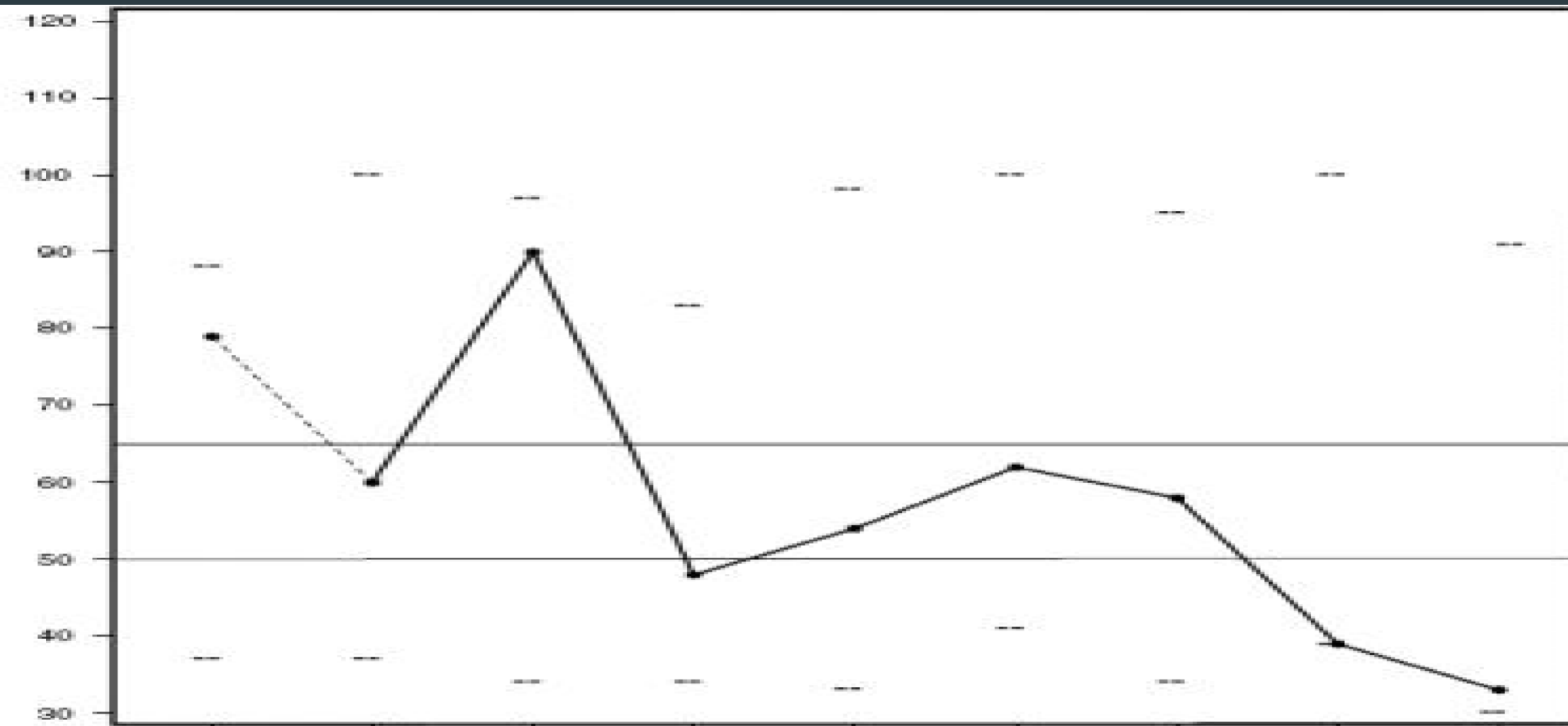
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- ❓ **Step 3. Organize the Scales and Identify the Code Type:** For the purposes of communicating scale scores, T scores rather than raw scores should be used. Code types can be determined by simply looking at the two highest scale elevations.
- ❓ **Step 4. Determine Profile Validity:** Assess the validity of the profile by noting the pattern of the validity scales. There are a number of indicators suggesting invalid profiles. The original validity indicators of the MMPI included "cannot say," L, F, and K.
- ❓ The basic patterns include a defensive style in which pathology is minimized (elevated L and/or K on the MMPI-2 and MMPI-A),
- ❓ an exaggeration of pathology (elevated F and/or Fb on the MMPI-2 or F, F1, or F2 on the MMPI-A),
- ❓ or an inconsistent response pattern (elevated VRIN or TRIN).

Graph Plotting according to the profile



For Example, Meaning Of Various Elevations On The (Pa) Paranoia Scale As Follows:

- ❑ T = 27-44 examinee may be stubborn, touchy, or difficult
- ❑ T = 45-59 no undue sensitivity and adequate regard for others
- ❑ T = 60-69 increasing probability of rigidity and oversensitivity
- ❑ T = 70-79 rigid, touchy, projects blame and hostility
- ❑ T = 79-100 frankly delusional paranoid features may be present
- ❑ The configural approach

- ❓ **Step 5. Determine Overall Level of Adjustment:** Note the number of scales over 65 and the relative elevation of these scales. The degree to which F is elevated can also be an excellent indicator of the extent of pathology (assuming that it is not so high as to indicate an invalid profile).
- ❓ **Step 6. Describe Symptoms, Behaviors, and Personality Characteristics:** This step represents the core process in interpretation. Mild elevations on individual scales (T = 60-65) represent tendencies or trends in the individual's personality. Interpretations should be treated cautiously with the more extreme descriptors being deleted or rephrased to represent milder characteristics. Scores in this range on the MMPI-A are highlighted by shading, thereby designating a marginal or transitional zone between normality and pathology.

- ❑ Step 7. Provide Diagnostic Impressions: useful in extracting relevant information for diagnosis.
- ❑ Step 8. Elaborate on Treatment Implications and Recommendations:
- ❑ to predict the client's likelihood of benefiting from interventions. This typically means elaborating on the person's strengths and weaknesses, level of defensiveness, ability to form a treatment relationship, predicted response to psychotherapy (note especially Es [ego strength] and TRT scales), antisocial tendencies, and level of insight.

Limitations

- ❓ if the test takers cannot interpret the items fully, the usefulness of the information obtained from the test may be limited. For example, if the test taker has cognitive limitations or learning disabilities or is a recent immigrant with a limited English-language knowledge, meanings of the MMPI-2 items may be obscured and the interpretation of the test results may be problematic
- ❓ Higher rate of can not say and faking makes protocol invalid



**STAY
COOL
AND
STAY
BLESSED**